

IMAGES IN CLINICAL MEDICINE

Erythema Multiforme



A PREVIOUSLY HEALTHY 22-YEAR-OLD MAN PRESENTED TO THE EMERGENCY department with a 5-day history of painful lesions on his hands, feet, and mouth, with accompanying fever and malaise. Two months earlier, he had had a similar episode. His temperature was 39.1°C. On physical examination, hemorrhagic crusting of the lips, an ulceration at the right commissure of the mouth, and shallow erosions on the tongue, palate, and buccal mucosa were observed (Panel A). Target lesions with central vesicles were also found on the palms (Panel B) and soles. Polymerase-chain-reaction testing of a swab sample from the lip ulceration was positive for herpes simplex virus (HSV) type 1. Testing was negative for human immunodeficiency virus and syphilis. A diagnosis of erythema multiforme associated with HSV infection was made. Erythema multiforme is an acute, immune-mediated mucocutaneous reaction that is most commonly triggered by infection. It may also be idiopathic or associated with medication. It can be accompanied by systemic symptoms in severe cases and may have a recurring course, as in this patient. Owing to discomfort associated with oral intake, the patient was treated with systemic glucocorticoids, along with therapeutic and then prophylactic antiviral therapy. Three weeks after presentation, the lesions had abated and had not recurred (Panels C and D).

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